



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : حنان محمد محمد اسماعيل
المحافظة : القاهرة
الحى / المدينة : الظاهر
رقم البطاقة : 26901010103261 رقم التليفون : 01111378265
العنوان : المعماري
العمر (تاريخ الميلاد) : 01-01-1969 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 06-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د.د. / د.د. / د.د. / د.د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: حنان محمد محمد اسماعيل
National ID: 26901010103261



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :

PM 01:17 05-07-2026

تاريخ وساعة الدخول :

---:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المريضة تعاني من ورم غير حميد بالبدن و تحتاج الي Supportive and palliative Treatment

الوقت :

---:--

التاريخ :

05-07-2026

الطبيب المعالج :

د/شيماء لاشين



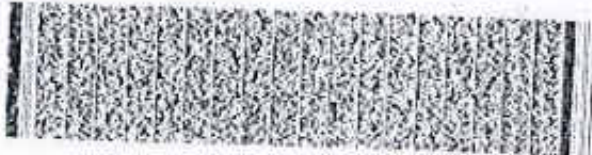
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حاصلة على ديون تجارة



التي مسلمة متروكة
 ماهر عبدالنور حسي على
 البطاقة مارية حتى ٢٠٣٠/٠٣/٠١



جمهورية مصر العربية
 بطاقة تحقيق الشخصية

جنان

محمد محمد اسماعيل

٢٢ عن محمود فهمي المعماري

القاهر - القاهر

٢٠٢٣/٠٣/٠١

٢٢٢٠١٠١٠١٠٣٢٢٢١

JW3674193

Dms

تطاليل واسماء

جمهورية مصر العربية
بطاقة تحقيق الشخصية

حنان
محمد محمد اسماعيل
٢٢ من محمود فهمي المصاوي
القاهرة - القاهرة

١٩٦٩/٠٧/٠١ ٢٦٩.٠١.٠١.٠٢٦٦٦

JW3674193

٢٠١٣/٠٣ ٢٦٩.٠١.٠١.٠٢٦٦٦

حاصلة على دبلوم تجارة

أنثى
متزوجة
مستلمة
ماهر عبدالرؤف حسني علي

البطاقة سارية حتى ٢٠٣٠/٠٣/٠١

١

Name: حسان محمد محمد		
MR. : 9827		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
wt 96 ht 162 sa2 postm mbc to lung liver hfl biosy er-ve pr -ve her2 3+ 2023 left ct4b n2 rec nact tchp on LEFT NRM 17/1/2024 REC KADCYLA X17 CYCLES PORT TILL CTS 10/2025 BILOBAR HFL LARGEST SEG 8 M 9.7X7.5 BIOPSIED ON 10/2025 EF 66 % 28/1/2026 MDT ON 11/1/2026 FOR TAXOL 160 HERCEPTIN Clinical Pharmacy Visit: Pt recieved W1+C1 Her on (27-1) Echo(27/1) 66% Today : For W2 Taxol 160 mg clinical pharmacy visit for w3 taxol 160 mg follow up liver function for c2 herceptin 440 mg and w4 taxol 160 mg .w5,6 pharmacy Pharmacy visit For week 5 taxol 160 mg labs 16/2 ... accepted Pharmacy visit For week 6 taxol 160 mg labs 16/2 ... accepted FOR C3 HERCPETIN IV, TAXOL WEEK 7 160 mg TAXOL W8 AND 9 PHARMACY echo 27/1 66% Pharmacy Visit For Week 8 Taxol 160 mg Labs 9/3 .. accepted MBC , today :W10 (WIH DOSE REDUCTION DUE TO PN 140 mg)+ C4 IV HERCEPTIN 440 (W11 &W12 IN PH). CT &ECHO ,,,PENDING Pharmacy Visit For W11 Taxol 140 mg Labs 30/3 .. accepted cts 4/2026 sd of pulmonary nodules 6 mm rd of multiple bilobar hfl largest seg 8 m 5.3x3.8 comp to 7.5 x9.5 cc picture of vertigo ef 63% 6/4/2026 pt has grade 2 pn for supp tt and assess pn next time and grade 2 onychomycosis for mri brain for c5 herceptin 440 iv and w14 taxol 140 Pharmacy Note For W13 Taxol 140 + C5 Herceptin 440 IV Labs 18/4 .. accepted Pharmacy Visit Wt: 96, Ht: 162, BSA: 2 FOr W15 Taxol 140 (DR due to PN gd 2) Labs 18/4 ... accepted mri brain pending for c6 herceptin 440 w16 taxol 140 w17 ,18 ph		
تحريراً في ,		
	التاريخ :	19-09-2026
	الوقت :	
	الطبيب المعالج :	dina makram
يعتمد ,		

Pharmacy Visit For C6 Herceptin 440 mg (didnt recieved at 12/5/2026)

Pharmacy Visit: Wt: 96, Ht: 162, BSA: 2 For W17 Taxol 140 (DR due to PN gd 2) Labs ... accepted

Pharmacy Visit For W18 Taxol 140 (DR) Labs 10/5 ... accepted

MBC FOR TAXOL W19 / HERCEPTIN 440 MG IV TODAY AFTER LABS CHECK NEXT TWO VISIT 9/6/2026 AND 16/6/2026 (W20 , 21 TAXOL) MRI DATED 21/5/2026 : MULTIPLE BIFRONTAL FOCI (ISCHEMIC)

mri brain 2ischemic foci for neuro cons , ,ef 63% 6/4/2026 .grade 2 pn for supp tt .for c8 herceptin 440 w22 taxol 140 mg w23,24 ph then new cts

Pharmacy Note for W23 Taxol 140 mg Labs 22/6 ... accepted

cc grade 2 pn ,ef 66% 13/7 for c9 herceptin 440 w25 taxol 115 (20 % DR) W26 ,27 PH FOR NEW CTS

Pharmacy Visit Wt: 96, Ht: 162, BSA: 2 For W26 Taxol 115 (DR due to gd 2 PN) Labs 13/7 ... accepted

ef 13/7/2026 66% cts on 21/7/2026 rd of pulmonary nodules 2 mm regressive disease in no and size of bilobar hfl largest in seg 8 m 3.5x2.3 comp to 5.3x3.8 for c10 herceptin 440 w28 taxol 140 w29 ,30 ph

Pharmacy Visit BSA: 2 For Week 29 Taxol 115 mg Labs 1/8 ... accepted

grade 2 pn on cymbatex 60 no other toxicity \for w30 taxol 115 and c11 herceptin 440,w31 32 ph

Pharmacy Visit Wt: 96, Ht: 162, BSA: 2 For W32 Taxol 115 (DR) Labs 25/8 ... accepted

MDT FORM

111 10/2026 G 2692

Name	محمد	MRN	
Age	56y	ID	
Menopausal status	post	Phone	0111378265
NRS SCORE	PS 0	Marital status	97.1.1.1.3571
BMI		Family history	-ve
CO morbidity	Low	REASON OF MDT	Metastatic

Clinical Data

BX: IDC

R

L

ER: 0/8 PR: 3/8 HER 2: +3 Ki67:

Sonammammography: 223) pt received
NA CT TCHP x 6

Mets Workup: thirt MRM →

1PT2 4PM2 → received PORT
and TDM1 for 1 year (last cycle 11/2024)

Recommendations: and kept under follow up.
CT Chest (6-10-2025) → Newly developed
U lower lobar nodule 0.5 cm.
→ Rt (HFL) → BX → GATA3 +ve
ER 0/8
PR 0/8
HER2 +3

MDT OUTCOME & PLAN

CT CAP & Contrast as baseline → R
T to Rechallenge with trastuzumab

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name : الأمثانه / حنان محمد محمد اسماعيل	Request No. : 13003668
Patient No. : 29601010103261	Request Date : 27/11/2025
Path No. : 3694/25	Verification Date : 04/12/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 56 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right hepatic lobe focal lesion, in patient with history of left MRM.
Nature of Specimen: Tru-cut biopsy, Collected on 27/11/2025, at 4:30 pm, Cold ischemia time: 30 seconds.

Gross Examination:
- Multiple fragmented grey white tissue cores, the longest is 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER clone (1E2), PR clone (SP1), Her2 clone (4B5), GATA3 (L50-823).

Microscopic Examination:
* Histopathological examination reveals cores of tumor formed of malignant epithelial cells showing moderate nuclear pleomorphism and eosinophilic cytoplasm, arranged in clusters among desmoplastic stroma.
* IPT (Positive control showed appropriate reactivity):
- GATA3: Diffuse strong positive staining in tumor cells.
- ER: Negative nuclear staining in tumor cells (score 0/8).
- PR: Negative nuclear staining in tumor cells (score 0/8).
- Her2: Strong complete membrane staining in more than 10% of tumor cells (score 3+).

Diagnosis:
Right hepatic lobe focal lesion, Tru-cut biopsy:
Findings are compatible with metastatic carcinoma of breast origin.

IPT:
- GATA3: Strong positive staining in tumor cells.
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).

Verified by :
Dr.Duaa Mohamed



Patient Name : الأستاذ / حنان محمد محمد اسماعيل
Patient No. : 29601010103261
Path No. : 3694/25
Sex : Female
Age : 56 Y
Referring Doc :

Request No. : 13003668
Request Date : 27/11/2025
Verification Date : 04/12/2025
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr. Duaa Mohamed

Printing Date : 06/12/2025 12:42:31

Page 2 of 2

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
(DOCTHREPATHOLOGY REPORT)



Patient Name: Hanan Mohamed Mohamed Ismail	Date : 27/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 56 Year(s)
Medical ID: 2692/2023	Pat. ID: 47379

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM in

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is heterogeneously dense which may obscure small lesions (ACR C),
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed
- Right breast 12 o'clock hypoechoic solid masse measured 9x8mm.
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- Right breast 12 o'clock hypoechoic solid masse.(BIRADS 3)

Yours truly

Signature Dr. basma mohmaed

27/02/2025 11:28 AM

Name : Mrs HANAN MUHAMED MUHAMED

Date: 2 September 2025

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

-Mild enlarged size (16.5 cm at the right MCL), displaying homogenous bright echo pattern with regular outer surface.

- No focal lesions detected

- No intra hepatic biliary radicle dilatation.

- Portal vein is patent and not dilated.

Gall Bladder: -

- Partially contracted yet no stones or mud seen.

- CBD is not dilated.

Spleen normal size measures about 17 cm with homogenous echo pattern, no focal lesions.

Kidneys: -

- Are of normal size , well differentiated & with no gross stones, or space occupying lesion could be seen , no back pressure changes detected.

- Para-aortic and pancreatic area: concealed by gases

- No ascites or free fluid or collection seen.

Urinary Bladder: -

- Average filling with normal wall thickness with no stones or focal lesions.

- Uterus and both ovaries : rudimentary size with no pelvic solid or cystic lesions.

Opinion:

- Bright mild hepatomegaly ? fatty liver for lab correlation.
- Much exaggerated colonic gases.

Best regards

D^r/Alubahr Hejazi F.E.B.R

Patient Name: Hanan Mohamed Mohamed Ismail	Date : 06/10/2025
Exam : C.T CHEST	Age : 56 Year(s)
Medical ID: 2692/2023	Pat. ID: 47379

MULTI-SLICE CT SCAN OF THE CHEST REVEALED: -

A reported case of Left MRM

- Compared to the previous CT scan performed on 13-4-2025, the current study revealed:
- Evidence of Left MRM no underlying mass or collection
- Newly developed left upper lobe apical segment nodule with spiculated margins
- Still noted right upper lobe posterior segment small pulmonary nodules measure 5 mm
- Left upper and lingular subpleural reticulation /atelectasis likely post irradiation squeal
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Upper chest cuts showed right hepatic lobe hypodense focal lesion

Opinion:

Newly developed left lower lobar nodule.

Right hepatic lobe hypodense focal lesion for dedicated triphasic CT scan

Dr. Asser Ahmed

Dr. Asser Ahmed

Dr. Asser Ahmed

Signature Dr.Mohamed Hassan
09/10/2025 3:09 AM

Patient's Name: HANAN MOHAMED MOHAMED ISMAEL
Date: 2026-04-12
Referred by: Prof. Dr. CT CAP IV
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported left breast carcinoma , compared to last study dated 15 /12/2025

(I) Chest:

- ☐ Left mastectomy with clear operative bed , masses or collection
- ☐ Stationary course regarding few pulmonary nodules , the largest at right lower lobe 6mm
- ☐ Lingular thick atelectatic band .
- ☐ Left upper lobe and lingula subpleural reticulations .
- ☐ Clear both lung fields .no sizable pulmonary masses or consolidation patches .
- ☐ No pleural or pericardial collections seen.
- ☐ No gross cardiac abnormality.

(II) Abdomen and pelvis:

- ☐ The liver is enlarged in size showing homogenous low CT density , multiple bilobed hepatic focal lesions , with regressive course regarding the largest seen at segment VIII measures 5.3 x 3.8 cm compared to 7.5 x 9.5
- ☐ Patent and homogeneously enhancing portal

Patient's Name:

Date:

Referred by:

Hospital:

HANAN MOHAMED MOHAMED ISMAEL

2026-04-12

Prof.Dr. CT CAP IV

Goustave Roussey International Egypt

- ☐ No intra or extra hepatic biliary radicles dilatation.
- ☐ Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- ☐ Normal CT appearance of the spleen.
- ☐ Regular filling of the urinary bladder showing no stones, masses.
- ☐ Unremarkable other pelvic organs.
- ☐ No retro-crural, para-aortic or pelvic lymphadenopathy.
- ☐ No ascites.
- ☐ No worrisome lytic or sclerotic bony lesions.
- ☐ No vertebral structural collapse.

OPINION:

- Regressive course regarding the hepatic focal lesions.
- Rather stationary rest of the study.

Dr.Reham Khedr, MD

Medical No. : 6337

Patient's Name: Hanan Mohammed Mohammed Ismail

Date: 2026-09-27

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



Findings:

- The liver is of enlarged size with regressive course as regards the number and the size of the few bilobar hepatic focal lesions largest is seen at segment VIII measuring 3x2.3 compared to 3.5x2.3cm with no appreciable enhancement.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Opinion:

- Regressive course as regards the size and the number of the few bilobar hepatic focal lesions.
- Dynamic MR liver is recommended if warranted

Regards, Dr. Ahmed Rezk, MD.

Medical No. : 6337
Patient's Name: Hanan Mohammed Mohammed Ismail
Date: 2026-09-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

جوستاف
رؤسي
الانساني
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POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION:-

Clinical History:

- Left MRM, compared to last study dated 21/7/2026

Findings:

- Still clear left mastectomy bed.
- Clear right breast and both axillae.
- Regressive course as regards the size and the number of bilateral scattered pulmonary nodules largest seen at the right upper lung lobe measuring 2mm
- Still noted few lingular and left upper lung lobe reticulations.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

OPINION:

- Still clear left mastectomy bed.
- Clear right breast and both axillae.
- Regressive course as regards the size and the number of bilateral scattered pulmonary nodules.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION:-

ECHO REPORT

NAME :

DATE: 13 / 7 / 2026

ID NUMBER : 5790 / 0.57

SEX : ♀ AGE: 56

Patient is Tachycardic

EF	66%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	2.8	3-3.7cm
LVEDS		2-4cm	LA	4.0	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	1.0	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

normal

LV Wall Thickness

normal

LV Diastolic Function

DD grade I

SWMA

0.55-1.10

LA and Ao Root Dimensions

normal

Mitral Valve

normal

Aortic Valve

normal

Tricuspid Valve

1.1/1.1 TR

Pulmonary Valve

normal

Right Side and Systolic Function

normal

Intracardiac Masses and Effusion

normal

DR.

Dr. Hisham

Patient's Name:

Hanan Mohammed Mohammed Ismail

Date:

2026-07-21

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION:-

Clinical History:

- Left MRM, compared to last study dated 12/4/2026

Findings:

- Still clear left mastectomy bed.
- Clear right breast and both axillae.
- Regressive course as regards the size and the number of bilateral scattered pulmonary nodules largest seen at the right lower lung lobe measuring 2mm compared to 6mm in previous study.
- Still noted few lingular and left upper lung lobe reticulations.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

OPINION:

- Still clear left mastectomy bed.
- Clear right breast and both axillae.
- Regressive course as regards the size and the number of bilateral scattered pulmonary nodules.

Patient's Name:
 Date:
 Referred by:
 Hospital:

Hanan Mohammed Mohammed Ismail
 2026-07-21
 Prof.Dr. no doctor
 Goustave Roussey International Egypt

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION:-

Findings:

- The liver is of enlarged size with regressive course as regards the number and the size of the few bilobar hepatic focal lesions largest is seen at segment VIII measuring 3.5x2.3cm compared to 5.3x3.8cm.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidney's with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suparenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Opinion:

- Regressive course as regards the size and the number of the few bilobar hepatic focal lesions.

Dr. Mai Shaaban MD

2023

GUSTAVE/ROUSSY
INTERNATIONAL
HOSPITAL

جوستاف روستي
التشخيصي
مركز

ECHO REPORT

NAME: *Mr. Mohamed*

DATE: 6/4/2022

ID NUMBER: 2692/2023

SEX: ♀ AGE: 56

EF	63%	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	3.1	3-3.7cm
LVEDS		2-4cm	LA	3.8	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	1.1	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions *normal*

LV Wall Thickness *normal*

LV Diastolic Function *DD grade I*

SWMA *normal*

LA and Ao Root Dimensions *normal*

Mitral Valve *Tild MR*

Aortic Valve *normal*

Tricuspid Valve *Tild TR*

Pulmonary Valve *normal*

Right Side and Systolic Function *normal*

Intracardiac Masses and Effusion *normal*

DR. *Jelwip*

Patient's Name: HANAN MOHAMED MOHAMED ISMAEL
Date: 2026-04-12
Referred by: Prof.Dr. CT CAP IV
Hospital: Goustave Roussey International Egypt

- ☐ No intra or extra hepatic biliary radicles dilatation.
- ☐ Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- ☐ Normal CT appearance of the spleen .
- ☐ Regular filling of the urinary bladder showing no stones, masses .
- ☐ Unremarkable other pelvic organs.
- ☐ No retro-crural, para-aortic or pelvic lymphadenopathy.
- ☐ No ascites.
- ☐ No worrisome lytic or sclerotic bony lesions .
- ☐ No vertebral structural collapse .

OPINION:.

- Regressive course regarding the hepatic focal lesions .
- Rather stationary rest of the study .

Dr.Reham Khedr, MD

Patient's Name:

HANAN MOHAMED MOHAMED ISMAEL

Date:

2026-04-12

Referred by:

Prof.Dr. CT CAP IV

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported left breast carcinoma , compared to last study dated 15 /12/2025

(I) Chest:

- ☐ Left mastectomy with clear operative bed , no masses or collection
- ☐ Stationary course regarding few pulmonary nodules , the largest at right lower lobe 6mm .
- ☐ Lingular thick atelectatic band .
- ☐ Left upper lobe and lingula subpleural reticulations .
- ☐ Clear both lung fields .no sizable pulmonary masses or consolidation patches .
- ☐ No pleural or pericardial collections seen.
- ☐ No gross cardiac abnormality.

(II) Abdomen and pelvis:

- ☐ The liver is enlarged in size showing homogenous low CT density , multiple bilobar hepatic focal lesions , with regressive course regarding the largest seen at segment VIII FL measures 5.3 x 3.8 cm compared to 7.5 x 9.5 cm .
- ☐ Patent and homogenously enhancing portal vein.

ECHO REPORT

NAME: Dr. Hani DATE: 5 / 10 / 2026

D NUMBER: / SEX: AGE: 56

EDD	64.1	55-75%	PWT	1.0	0.6-1.1cm
EDD	4.8	3.5-5.8cm	AO.R	2.4	3-3.7cm
ESD		2-4cm	LA	3.7	2-4cm
		25-45%	RV		0.7-2.3cm(supine)
MT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions normal
 LV Wall Thickness normal
 LV Diastolic Function DD grade I
 SWMA 0.8-1.4
 LA and Ao Root Dimensions normal
 Mitral Valve normal
 Aortic Valve normal
 Tricuspid Valve normal
 Pulmonary Valve normal
 Right Side and Systolic Function normal
 Intracardiac Masses and Effusion 0

DR.
Dr. Hani